

OREGON STATE HOSPITAL

POLICY

SECTION 10: Medications **POLICY: 10.003**

SUBJECT: Medication Crush

POINT PERSON: Chief Medical Officer

APPROVED: Sara Walker, MD

DATE: SEPTEMBER 20, 2024


Interim Superintendent

SELECT ONE: ☐ New policy

☒ Minor/technical revision of existing policy

☐ Reaffirmation of existing policy

☐ Major revision of existing policy

I. PURPOSE AND APPLICABILITY

- A. Oregon State Hospital (OSH) has the responsibility to provide safe care to patients. According to accepted standards of medication management practices, OSH will require crushing of medications that have moderate to high risk for abuse in order to minimize the potential for diversion. This policy applies to all OSH programs.
- B. This policy applies to all staff who provide medication to OSH patients.

II. POLICY

- A. All medications listed in this policy must be crushed unless specifically labeled or noted by the Pharmacy Department as DO NOT CRUSH (e.g., sustained release, special formulations).
 - 1. All immediate-release opioid analgesics including, but not limited to:
 - a. Oxycodone products (e.g. Oxy IR, Percocet)
 - b. Hydrocodone products (e.g. Norco)
 - c. Morphine products
 - d. Tramadol products (e.g. Ultram)
 - e. Codeine products (e.g. Tylenol #3)
 - f. Hydromorphone products (e.g. Dilaudid)
 - g. Methadone
 - h. Oxymorphone products (e.g. Opana)
 - 2. All benzodiazepines including, but not limited to:

- a. Lorazepam (Ativan)
 - b. Clonazepam (Klonopin)
 - c. Diazepam (Valium)
 - d. Temazepam (Restoril)
 - e. Chlordiazepoxide (Librium)
3. Non-benzodiazepine sedative/hypnotic agents (e.g. “Z-drugs”) including, but not limited to:
 - a. Zolpidem (Ambien)
 - b. Eszopiclone (Lunesta)
 - c. Zaleplon (Sonata)
4. Gabapentinoids including, but not limited to:
 - a. Gabapentin (Neurontin)
 - b. Pregabalin (Lyrica)
5. Stimulants (immediate release) including, but not limited to:
 - a. Amphetamine products (e.g. Adderall, Dexedrin)
 - b. Methylphenidate products (e.g. Ritalin)
 - c. Modafinil (Provigil)
6. Miscellaneous (immediate release) drugs with abuse potential at OSH including, but not limited to:
 - a. Quetiapine (Seroquel)
 - b. Bupropion products (e.g. Wellbutrin)
- B. Physicians and nurse practitioners may write orders to crush other medications not included on the list above as appropriate (e.g., “Crush Acetaminophen”).
- C. If a physician/ nurse practitioner believes a patient requires an exception for the crush order, they must receive approval from the CMO.
 1. Exceptions may only be granted if all other medication options have been exhausted (ex. liquid form, IM, oral disintegrating) and if Occupational Therapy (OT) or Speech-Language Pathology (SLP) have recommended an exception due to sensory or swallowing concerns. Exceptions will not be granted based on patient preference alone.
- D. Nursing staff must perform careful mouth checks for those patients that have an exception and immediately report any behaviors of concern to the physician/nurse practitioner and interdisciplinary treatment team.
- E. OSH follows all applicable regulations, including federal and state statutes and rules; Oregon Department of Administrative Services (DAS), Shared Services, and Oregon Health Authority (OHA) policies; and relevant accreditation standards. Such regulations supersede the provisions of this policy unless this policy is more restrictive.

- F. Staff who fail to comply with this policy or related policy attachments or protocols may be subject to disciplinary action, up to and including dismissal.

III. DEFINITIONS

- A. "Staff" includes employees, volunteers, trainees, interns, contractors, vendors, and other state employees assigned to work at Oregon State Hospital (OSH).

IV. RELATED OSH POLICIES AND PROTOCOLS

Nursing Services Department Protocol 2.060 Medication Administration

6.055 Medication management